



- Continue to address specific differentials associated with signs, symptoms, and/or dysrhythmia.

Consult Online Medical Control As Needed



Cardiac Arrest: Treatable Causes

Pearls:

- Providers should not hasten departure after obtaining ROSC. Instead, focus on stabilizing the patient and ensuring that the airway is appropriately managed. If rearrest occurs, it will be in the first few minutes.
- Refer to drug formulary charts for all medication dosing for both adults and pediatric patients.
- In order to be successful in adult or pediatric arrests, a cause must be identified early and corrected. Resuscitation should include targeted therapies to address the underlying cause of the arrest.
- Respiratory arrest is a common cause of pediatric cardiac arrest. Unlike adults early oxygen and ventilation is critical.
- In most cases pediatric airways can be managed by basic interventions.
- Effective CPR is critical: 1) Push hard and fast at appropriate rate 2) Ensure full chest recoil 3) Minimize interruptions in CPR. Pause CPR < 10 seconds only.
- Effective CPR and treatment of underlying causes are the keys to successful resuscitation.
- Prolonged cardiac arrests may lead to tired providers and decreased compression quality. Ensure compressor rotation, summon additional resources as needed, and ensure provider rest and rehab during and post-event.
- For pediatrics use volume control device (IV Burette) for Dextrose and Fluid infusions
- Always quickly confirm asystole in more than one lead and, trouble shoot for Equipment settings/problems
- Reassess and document ETT/BIAD placement continuously after every move and at transfer of patient care.
- Continuous ETCO₂ should be initiated as soon as practicable.
- Calcium and sodium bicarbonate should be given early if hyperkalemia is suspected (renal failure, dialysis)
- Continue to use primary monitor for all event recording and data capture.
- All monitor event data and recordings are uploaded into e-PCR.
- Ultrasound to determine cardiac wall motion at pulse check; DO NOT interrupt compressions for ultrasound.